

Riverbend Medical group



4275 Willow Creek Drive
Suite 204
Springfield, IL 62704 Fax: (217) 555-0176

phone: (217) 555-0134

Patient Information

Name:

Mary M

Email:

Marym23@gmail.com

Kindly fill the box

Personal Details Name : Mary M

Gender:Female

Age: 45

DOB: 4/04/1974

DOS: 12/28/2024

Provider Name: Riverbend Medical Group



Provider details:4275 Willow Creek Drive

Suite 204

Springfield, IL 62704

Phone: (217) 555-0134

Fax: (217) 555-0176

Smoking and Drinking history

				Comments
Have you smoke tobacco ?	☐ Yes	☐ No	☐ occasional	
How many packs per day did you or do you smoke	☒ <1	☐ 1	☐ >2	
if you have quit smoking,at what age did you quit			Total pack-years smoking?	more than 50
How often do you have a drink containing alcohol?	☐ Never	☒ Monthly or less	☐ Unknown	
how many drinks containing alcohol do you have on a typical day when you drink?	☐ 1 or 2	☒ 3 to 4	☐ Unknown	
Alcohol used within last 3 months?	☒ Yes	☐ No	☐ Unknown	
Are you actively participating in an alcohol treatment program	☐ Yes	☒ No	☐ N/A	

Labs & Procedures

Exam Date:	Procedure			Results
4/03/2024	☒ A1c	☐ PAD	☐ Egfr	6.7
4/03/2024	☐ A1c	☒ PAD	☒ Egfr	Unknown
6/03/2024	☒ A1c	☐ PAD	☐ Egfr	6.9
10/05/2024	☒ A1c	☐ PAD	☐ Egfr	6.8-7.0

General Appearance & Station

Height(cm)	168
Weight(lbs)	152
BMI	
Does member have any obesity-related comorbid condition ?	

Member has been
diagnosed previously with
other cancer

☐ Yes

☒ No

how often Mammogram
follow up recommended

☐ Hardly

☒ Once

☐ Monthly

☐ More

Chief Complaint

- The Chief Complaint is: Follow-up for depression and HTN.
- Patient is here for follow-up of the chronic conditions.
- Essential hypertension.

Medical History

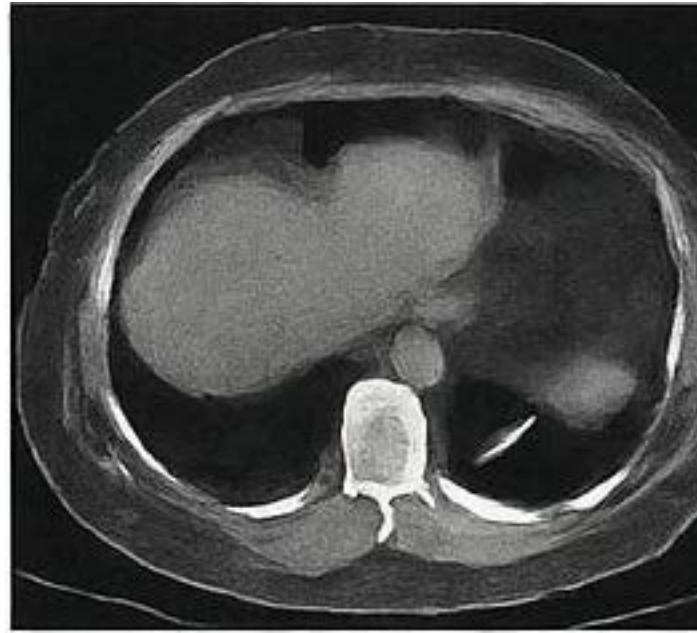
Following the stool DNA test, the patient received a positive result indicating the presence of genetic abnormalities and biomarkers associated with colorectal cancer.

Based on the positive stool DNA test result, the patient's healthcare provider performed a large bowel endoscopy on 11/12/2023, ensuring a comprehensive evaluation of the patient's colon and rectum.

Another procedure was carried out on 12/23/2023 in accordance with standard medical protocols, utilizing a flexible tube with a camera to visualize the entire colon and rectum during the examination. Abnormal tissues and polyps were identified, and appropriate measures were taken.

In 2024, she received a diagnosis of colorectal cancer after a positive fecal occult blood test, leading to a screening and polypectomy. During the colorectal screening using a flexible tool, multiple polyps were discovered, with one exhibiting high-grade dysplasia. She is currently under surveillance for potential recurrence.

The patient underwent a surgical procedure in which the entire colon (large intestine) was removed one day after Martin Luther King Jr. Day in 2024. This procedure was performed due to medical conditions that severely affected the colon. In this case of advanced colorectal cancer, surgery was necessary to remove the cancerous tissue and prevent further spread. The patient is scheduled for a colonoscopy on 10/01/2024.



Colon Scan
Inconclusive. stispecte

GYN History

The patient presented with heavy and prolonged menstrual bleeding (menorrhagia) and persistent pelvic pain, which significantly affected her quality of life. After thorough evaluation, the patient underwent cervical endocervical cytology, performed on 1/12/2022. It was determined that the patient had the following medical conditions that necessitated the performance of abdominal surgery of the uterus with surrounding structures after three days: **Uterine Fibroids:** Multiple non-cancerous growths (fibroids) in the uterus causing enlargement and excessive bleeding. **Adenomyosis:** A condition where the tissue lining the uterus (endometrium) grows into the muscular wall of the uterus, leading to pain and heavy bleeding. **Chronic Pelvic Inflammatory Disease:** Persistent inflammation and infection of the pelvic organs, contributing to pelvic pain and discomfort. Additionally, since 2022, she has been dealing with polycystic ovarian syndrome and has initiated treatment with metformin to address her symptoms. **Incision:** A transverse incision was made in the lower abdomen to access the pelvic organs. **Uterine Dissection:** The surgeon carefully dissected the uterus from surrounding structures, including ligaments and blood vessels. **Bilateral Salpingo-Oophorectomy:** During the procedure, the surgical team observed the following: **Multiple Uterine Fibroids:** Several fibroids of varying sizes were present in the uterine wall. **Enlarged Uterus:** The uterus was enlarged due to the presence of fibroids and adenomyosis. **Adhesions:** Pelvic adhesions were observed, which could contribute to the patient's pelvic pain. In addition to the uterus, both Fallopian tubes and ovaries were removed to prevent future complications and reduce the risk of ovarian cancer.

Closure: The incision was meticulously closed using sutures. Following the abdominal surgery on the uterus, the patient was closely monitored in the recovery room for the appropriate period. Vital signs, pain levels, and surgical site were regularly assessed

Retinal Diagnostic Report

Southdale Eye clinic

Patient information

Name: Mary M
DOB:4/04/1974
Medical record
number: 098-67-4567

Date:5/30/2024
Gender: Female
Referring Physician:DR Sandhya

Ordering Provider:Dr.Jess L. Boysen, M.D.
Southdale Eye clinic
Performing Location:
6533 Drew Avenue South
Edina, MN 55435

Dx Date	Dx	Description	eye	Care plan
05/28/2024	E119	Type 2 diabetes mellitus without complications		Stressed importance of proper diet and exercise, Send letter to PCP , Stressed importance for compliance to medications.
05/28/2024	H35,033	Hypertensive retinopathy, bilateral		Stressed importance of proper diet. Educated patient on findings. Advised patient to continue taking all medications prescribed by PCP or specialist.
05/28/2024	H52.4	Presbyopia	OU	Advised patient to update spectacle lenses. Adaption to refractive correction expected. Glasses to be whom full-time.

Diagnosis:

Dr. Jess

05/28/2024

Disposition:

Patient is pleasant and sociable.

Orientation: Patient is fully alert to time, place, and person. adaptation to refractive correction is expected.

Diagnosis of hypertensive retinopathy is positive.

.Eye exam report is attached with this document



Reason for Visit: Severe shortness of breath, chest tightness, and dizziness.

Clinical Summary:

Past Medical History: Hypertension, Type 2 Diabetes, Asthma (moderate persistent), Obesity, Hyperlipidemia.zz

Medications:

Systemic: No fever, chills, or night sweats. Cardiovascular: No chest pain or discomfort. Pulmonary:

No dyspnea or cough.

Gastrointestinal: No nausea, vomiting, diarrhea, or constipation. Social: No recent travel to a foreign country.

BP was recorded as 156/90 mmHg, followed by 138/84 mmHg (as reported by the patient) recorded on August 3 2024. On August 8, 2024, the patient was brought to the ER with a blood pressure reading of 140/90 mmHg in the left arm. Two days after the ER visit, the patient reported that their blood pressure at home had been consistently ranging from 144-151/83-88 mmHg. As a result, the patient was advised to consult with their healthcare provider for blood pressure monitoring on August 20, 2024, but they later called and cancelled the appointment. The day after canceling, the patient visited the office and recorded a blood pressure of 140/90 mmHg using a non-digital blood pressure machine. Pulse Rate: 94 bpm.

Pulse Rhythm: Regular.

Respiration Rate :20 per min

Body Surface Area: 1.66m2.

SPO2: 94%.O2Device: None (Room Air). FiO2: 21%.

Jess.t